

1. A 6-year-old boy is brought to the physician by his parents because he continues to wet the bed approximately five times weekly. He has never had a sustained dry period longer than 7 days. Most of the "accidents" occur during the first 3 hours of the night and occasionally during dreams in which he imagines himself urinating. He is afraid to go to friends' houses for sleepovers because of this problem. He worries that his friends would make fun of him if they knew, and he frequently feels bad about himself "because I pee the bed like a baby." He was diagnosed with type 1 diabetes mellitus 1 year ago. It is well controlled with insulin, and he is compliant with the medication regimen. Examination shows no abnormalities. Which of the following is the most likely explanation for this patient's bed-wetting?
- ☐ A) Generalized anxiety disorder
 - ☐ B) Oppositional defiant disorder
 - ☐ C) Primary enuresis
 - ☐ D) Social phobia
 - ☐ E) Urinary overflow incontinence secondary to diabetes mellitus



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2. A 77-year-old woman is brought to the physician by her daughter because of a 3-year history of worsening forgetfulness. The patient says that she has not had any problems and does not know why her daughter is so concerned. She is able to recall zero of three objects after 5 minutes. She has significant deficits in praxis, verbal category fluency, and naming. The remainder of the examination shows no abnormalities. Results of laboratory studies are within the reference range. Which of the following neurotransmitters is most closely associated with the pathophysiology of this patient's illness?

- ☐ A) Acetylcholine
- ☐ B) γ -Aminobutyric acid
- ☐ C) Dopamine
- ☐ D) Glutamate
- ☐ E) Norepinephrine
- ☐ F) Serotonin



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3. A 72-year-old man with Parkinson disease is brought to the physician by his wife for a follow-up examination 6 weeks after pramipexole was added to his regimen of carbidopa-levodopa to manage increasingly severe bradykinesia. The patient reports improved movement and greater stability with walking. His wife says that he has become increasingly suspicious of her and has accused her of infidelity. Last week, she awoke during the night and found him sitting in the living room with his hunting rifle on his knees. The patient had insisted that a prowler or "her paramour" was sneaking around the house, and he intended to "teach him a lesson he won't have to learn twice." The patient has no history of psychiatric illness, but his wife says he has seemed depressed and has had decreased appetite and motivation during the past 4 weeks. The patient just shrugs his shoulders in response to his wife's comment. He has hypertension controlled with lisinopril and hyperlipidemia controlled with lovastatin. His blood pressure is 138/84 mm Hg. Examination shows less rigidity and increased arm swing compared with examination results 6 weeks ago. On mental status examination, he is cooperative and superficially polite. He has a mildly dysphoric mood and restricted affect. Which of the following is the most likely explanation for this patient's change in mental status?
- ☐ A) Adverse effect of pramipexole
 - ☐ B) Delusional disorder
 - ☐ C) Major depressive disorder
 - ☐ D) Paranoid personality disorder
 - ☐ E) Progression of Parkinson disease



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4. A 25-year-old woman is brought to the emergency department by her mother, who reports that the patient has been threatening her at home during the past 2 weeks. The patient has a 5-year history of schizophrenia. Her psychiatric symptoms had been "fairly" well controlled with oral quetiapine for 6 months, but she reports that she stopped taking her medication 6 weeks ago because it was sedating her. Physical examination shows no abnormalities. Mental status examination shows an agitated mood and a guarded affect. She appears to be responding to internal stimuli. Which of the following is the most appropriate next step in pharmacotherapy?

- ☐ A) Increase the dosage of quetiapine
- ☐ B) Switch from quetiapine to amitriptyline
- ☐ C) Switch from quetiapine to aripiprazole
- ☐ D) Switch from quetiapine to bupropion
- ☐ E) Switch from quetiapine to chlorpromazine



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5. An 86-year-old woman is brought to the physician because of confusion, forgetfulness, and lethargy for 6 weeks. She lives by herself and has been functioning independently. She has hypertension, hypercholesterolemia, and osteoarthritis of the lumbar spine and hips. Her drug regimen includes hydrochlorothiazide, captopril, naproxen, gemfibrozil, and lovastatin. Her temperature is 37°C (98.6°F), pulse is 87/min, and blood pressure is 110/70 mm Hg. Reflexes bilaterally are brisk, and muscle strength in the upper and lower extremities is excellent. On mental status examination, she is oriented to person but not to time and place and is unable to remember three objects after 2 minutes; she cannot perform serial sevens and is frustrated when she cannot answer proverb or judgment questions. Laboratory studies show:

Hematocrit	44%
Leukocyte count	9600/mm ³
Serum	
Na ⁺	121 mEq/L
Cl ⁻	86 mEq/L
K ⁺	3.0 mEq/L
HCO ₃ ⁻	28 mEq/L
Ca ²⁺	9.6 mg/dL
Urea nitrogen	20 mg/dL
Creatinine	2.0 mg/dL

Toxicology screening is negative. A CT scan of the head shows mild diffuse cerebral atrophy with no lesions. An x-ray of the chest shows normal findings. Which of the following is the most likely cause of the change in mental status?

- ☐ A) Adverse effect of captopril therapy
- ☐ B) Adverse effect of hydrochlorothiazide therapy
- ☐ C) Adverse effect of lovastatin therapy
- ☐ D) Cerebral infarction
- ☐ E) Dementia, Alzheimer type



6. A 35-year-old woman, gravida 2, para 2, has had episodes of anxiety, bloating, fatigue, irritability, and breast tenderness for the past 2 years. Her symptoms begin about 10 days before menses and cease with the onset of menses. Physical examination is within normal limits. Which of the following is the most appropriate next step in patient care?

- ☐ A) Reassurance and reexamination in 6 months
- ☐ B) Instruction to keep a diary of symptoms for the next two menstrual cycles
- ☐ C) Behavior modification therapy
- ☐ D) Measurement of serum thyroxine (T_4), thyroid-stimulating hormone, and prolactin concentrations
- ☐ E) Therapeutic trial of a benzodiazepine



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7. A 48-year-old woman is brought to the emergency department by her daughter 2 hours after she fell at home. Physical examination shows ecchymoses over the left hip and left shoulder but is otherwise normal. During evaluation, the daughter pulls the physician aside and explains that her mother has drunk excessively over the past 12 years. She frequently consumes several bottles of wine alone at home each day. She also reports that her mother has been forgetting to take cooking pots off the stove. When the physician continues with the evaluation, the patient behaves as if she has never met the physician before now. On cognitive examination, she recalls zero out of three objects after 5 minutes. Which of the following is the most likely cause of this patient's cognitive decline?

- ☐ A) Creutzfeldt-Jakob disease
- ☐ B) Dementia, Alzheimer type
- ☐ C) Korsakoff psychosis
- ☐ D) Multi-infarct (vascular) dementia
- ☐ E) Wernicke encephalopathy



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8. A 5-year-old girl is brought to the physician by her father as a new patient for a well-child examination. On entering the physician's office, the patient immediately climbs onto the physician's lap and begins talking with the physician. Her father says that his daughter has shown poor learning ability and has outbursts in school. He also says she is highly sensitive to sound. Examination shows a small, upturned nose, small chin, and long philtrum. Muscle tone is decreased. Which of the following is the most likely diagnosis?

- ☐ A) Cornelia de Lange syndrome
- ☐ B) Down syndrome
- ☐ C) Prader-Willi syndrome
- ☐ D) Smith-Magenis syndrome
- ☐ E) Williams syndrome



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9. A 77-year-old woman comes to the physician at the request of an attorney prior to writing an advance directive. She says that she wants to have an advance directive in order to avoid putting an unnecessary burden on her children. She has no history of serious illness and takes no medications. She immigrated to the USA from Poland 20 years ago. She has only an elementary school education. She was employed in a cleaning service for a hotel but stopped working 5 years ago. She lives alone and is able to perform activities of daily living independently. She participates in social activities at her church. All of her close friends are also from Poland. Physical examination shows no abnormalities. On mental status examination, she is nervous. She is alert and is oriented to person. She knows she is in the physician's office but does not recall the street name correctly. She knows the month and day of the week but is not sure of the exact date. She knows her address. She can recall two of three words after 5 minutes. She can subtract 7 from 100 using her fingers but is unable to subtract 7 from 93. She is unable to spell the word "world" backwards. She can draw the hands on a clock to show 10 minutes to 2 o'clock and can slowly copy a drawing of two intersecting pentagons. She demonstrates that she understands the meaning of a standardized story. Which of the following is the most appropriate next step with respect to this patient's advance directive?
- ☐ A) Discussion of her preferences for care
 - ☐ B) Second examination in 1 week
 - ☐ C) Repeat her mental status examination with an interpreter
 - ☐ D) Neuropsychological testing
 - ☐ E) CT scan of the head



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10. Six hours after undergoing an appendectomy, a 47-year-old man reports a moderately severe, diffuse headache. He has no other symptoms. He has not ingested food or drink for 2 days. His pulse is 90/min, and blood pressure is 110/70 mm Hg. Neurologic examination shows no abnormalities. Which of the following is the most likely diagnosis?

- | | |
|---|--|
| ☐ A) Bacterial meningitis | ☐ G) Occipital neuralgia |
| ☐ B) Brain abscess | ☐ H) Post-traumatic headache |
| ☐ C) Caffeine withdrawal | ☐ I) Subdural hematoma |
| ☐ D) Cluster headache | ☐ J) Tension-type headache |
| ☐ E) Hypertensive encephalopathy | ☐ K) Trigeminal neuralgia |
| ☐ F) Idiopathic intracranial hypertension | |



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11. A 67-year-old man comes to the physician because of a 1-year history of difficulty sitting still for more than 1 hour. He says that he has a nearly intolerable sensation of restlessness in his legs when watching movies or during long flights. His symptoms resolve when he stands up and takes a few steps. He also has had difficulty falling asleep because of his symptoms and states that he thinks he moves around a lot once he falls asleep because his wife often complains that he kicks her in bed. He has no history of serious illness and takes no medications. He does not drink alcohol or use illicit drugs. Physical examination shows no abnormalities. Deep tendon reflexes are normal in all extremities. Sensation is intact. Which of the following is the most appropriate initial pharmacotherapy?

- ☐ A) Hydroxyzine
- ☐ B) Modafinil
- ☐ C) Ropinirole
- ☐ D) Trazodone
- ☐ E) Zolpidem



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12. A 37-year-old woman comes to the physician because her husband is concerned that she has been having difficulty remembering things around the house. During the past 6 weeks, she has been forgetting to do her usual household chores. She does not think that she has a problem. She says that she does her best but often feels fatigued, especially when moving around a lot. She has had numbness of both legs and intermittent blurred vision during the past year. Physical examination shows pallor of the optic discs. Deep tendon reflexes at the knees and ankles are brisk bilaterally. On mental status examination, she has an upbeat mood and affect. She is oriented to person, place, and time. She has difficulty remembering the names of recent U.S. Presidents. There is no evidence of auditory or visual hallucinations. A T2-weighted MRI of the brain shows numerous small plaques in the periventricular white matter. A lumbar puncture is done. Which of the following findings is most likely on cerebrospinal fluid analysis?

- ☐ A) Decreased glucose
- ☐ B) Decreased total protein concentration
- ☐ C) Eosinophilocytosis
- ☐ D) Oligoclonal IgG bands
- ☐ E) No abnormalities



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13. A 57-year-old lawyer is brought to the emergency department by police after he interrupted a church service by loudly proclaiming that he is the Messiah. Police say that he became angry and physically confrontational when he was asked to leave and continued to insist that he was the Messiah when police intervened. On arrival in the emergency department, he is expansive and talks loudly. He appears well groomed. On questioning, he says he has never been sick and takes no medications. He does not drink alcohol or use illicit drugs. He refuses to allow a physical examination and becomes irritable with the nurses. He demands to see the person in charge of the hospital, noting he has figured out a way to save the hospital millions of dollars. On mental status examination, he talks rapidly and abruptly shifts from topic to topic. He reports hearing the voice of God telling him that He is pleased with him. Which of the following is the most likely diagnosis?

- ☐ A) Bipolar disorder
- ☐ B) Delusional disorder
- ☐ C) Dysthymic disorder
- ☐ D) Schizophrenia
- ☐ E) Schizotypal personality disorder



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14. A 32-year-old man is brought to the emergency department 2 hours after the onset of chest pain, palpitations, and agitation. He has a history of cocaine abuse and has been treated in the emergency department six times during the past 4 months for episodes of agitation and hallucinations. He has no other history of serious illness and takes no medications. On arrival, he is agitated but cooperative. His temperature is 37.8°C (100°F), pulse is 130/min, and blood pressure is 160/100 mm Hg. Examination shows no other abnormalities. Urine toxicology screening is positive for cocaine. An ECG shows new ST-segment depressions in the anterior leads, which were not found on ECGs obtained at previous visits. Which of the following medications is contraindicated in this patient?

- ☐ A) Aspirin
- ☐ B) Diazepam
- ☐ C) Diltiazem
- ☐ D) Heparin
- ☐ E) Metoprolol



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15. A 37-year-old woman comes to the physician because of a 4-month history of recurrent thoughts that she has sexually abused her 5-year-old daughter. She says that she examines her daughter's perineum several times daily looking for evidence of abuse. Examination of the child by her pediatrician has shown no evidence of abuse. The patient believes that she is going crazy. She reports that she was sexually abused as a child. Physical examination shows no abnormalities. On mental status examination, she appears tense, anxious, and mildly depressed. She cries as she describes her worrisome thoughts. There is no evidence of hallucinations. Which of the following is the most likely diagnosis?

- ☐ A) Acute stress disorder
- ☐ B) Delusional disorder
- ☐ C) Dissociative amnestic disorder
- ☐ D) Factitious disorder by proxy
- ☐ E) Obsessive-compulsive disorder
- ☐ F) Post-traumatic stress disorder



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16. A 6-year-old boy is brought to the physician for a well-child examination. His mother states that he has had increasingly impulsive behavior and aggressive outbursts during the past year. His kindergarten and first grade teachers expressed concern about his ability to follow directions and socialize with peers. At the age of 3 years, he received treatment for lead exposure. Language development is appropriate; toilet training milestones have been met. His mother states that he is a picky eater. He has no history of serious illness. His mother believes that he is angry about his parents getting a divorce. He is alert and fidgety. He is at the 5th percentile for height and weight. His pulse is 78/min, and blood pressure is 97/60 mm Hg. Examination shows no dysmorphic features. The thyroid gland is not enlarged. Pulmonary and abdominal examinations show no abnormalities. The upper and lower extremities are thin. Muscle tone is normal. Deep tendon reflexes are brisk. He is easily distractible, impulsive in observed play, and oppositional when his mother attempts to redirect him. Which of the following is the most appropriate next step in diagnosis?

- ☐ A) Behavioral rating scale by teachers
- ☐ B) Projective testing by school psychologist
- ☐ C) Thyroid function tests
- ☐ D) MRI of the brain
- ☐ E) EEG



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17. A 15-year-old boy is brought to the emergency department by paramedics 30 minutes after his friends noticed that he had become confused and unsteady. His friends reported that he "may have been trying to get high." Glucose and naloxone administered en route to the emergency department produced no change in his condition. On arrival, he is lethargic, and his speech is slurred. His temperature is 37°C (98.6°F), respirations are 12/min, and blood pressure is 150/95 mm Hg. His gait is ataxic. Results of urine toxicology screening are pending. A CT scan of the head shows no abnormalities. The patient falls asleep for 1 hour, and a second neurologic examination on awakening shows full alertness and a normal gait. Which of the following substances is the most likely cause of these findings?

- ☐ A) Alcohol
- ☐ B) Cocaine
- ☐ C) Heroin
- ☐ D) Marijuana
- ☐ E) Toluene



18. A 20-year-old woman comes to the physician with her husband because of increasing fatigue and social withdrawal. The patient reports that she has always worried about her appearance and how other people perceive her. She has difficulty shopping and states, "Every time I go into a grocery store, I feel like I might not get out alive." During the past 3 months, she has induced vomiting 6 to 10 times weekly when she feels "panicky" after eating "bad foods." Her husband reports that she becomes anxious and irritable when she has not exercised. She has not had a menstrual period for 6 months. Her husband expresses frustration with her problems and says that her obsessive worrying and poor self-image are ruining their marriage. She has no history of serious illness and takes no medications. She is thin and meticulously groomed. She is 165 cm (5 ft 5 in) tall and weighs 48 kg (105 lb); BMI is 18 kg/m². Her temperature is 37°C (98.6°F), pulse is 50/min, respirations are 14/min, and blood pressure is 102/68 mm Hg. Physical examination, including pelvic examination, shows no abnormalities. The patient states that her recent weight loss of 6.8 kg (15 lb) has made her feel less anxious and that she plans to lose more weight. Which of the following is the most likely diagnosis?

- ☐ A) Anorexia nervosa
- ☐ B) Body dysmorphic disorder
- ☐ C) Bulimia nervosa
- ☐ D) Generalized anxiety disorder
- ☐ E) Obsessive-compulsive disorder



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19. A 4-year-old girl is brought to the physician by her mother because of frequent temper tantrums and extreme irritability over the past 6 months. She was born at 35 weeks' gestation and had jaundice at birth. Her 17-year-old mother reports that her daughter refuses to listen to her and will not eat foods she previously ate. The mother appears sad and states that she is overwhelmed. The patient is at the 15th percentile for height and weight. Physical examination shows no abnormalities. She is very active during the examination. Which of the following is the most appropriate next step in management?

- ☐ A) Play therapy
- ☐ B) Parenting classes
- ☐ C) Measurement of serum albumin concentration
- ☐ D) Measurement of urine amino acid concentration
- ☐ E) MRI of the brain
- ☐ F) Methylphenidate therapy



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20. A previously healthy 57-year-old woman comes to the physician because of memory loss for 3 months. Examination shows a very sluggish patient with a flat affect who talks slowly and in a whisper. Initially, she replies to all mental status tests, "I don't know" or "I can't remember." As the physician shows a warm and supportive manner, her responses improve, so that eventually she recalls the main event in the news the previous evening and is able to recall two out of three objects after 5 minutes. She is told the third object. When retested just before leaving 10 minutes later, she recalls all three. Which of the following is the most likely diagnosis?

- ☐ A) Communicating hydrocephalus
- ☐ B) Concussion
- ☐ C) Dementia, Alzheimer type
- ☐ D) Herpes simplex encephalitis
- ☐ E) Hysterical amnesia
- ☐ F) Major depressive disorder with atypical features
- ☐ G) Transient global amnesia



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21. A 21-year-old college student is brought to the emergency department by her roommates because of a 4-day history of progressive generalized weakness and muscle cramps. She has been unable to eat solids for 2 days. Her last bowel movement was 4 days ago. Her roommates report that, during the past year, they have heard her vomiting in the bathroom after meals. Her last menstrual period was 3 weeks ago. The patient is oriented to person, place, and time and says that she feels tired. Her temperature is 36°C (96.8°F), pulse is 70/min, and blood pressure is 100/70 mm Hg. The abdomen is soft, and bowel sounds are decreased. Muscle strength is decreased throughout. Which of the following is the most appropriate next step in diagnosis?

- ☐ A) Arterial blood gas analysis
- ☐ B) Complete blood count
- ☐ C) Measurement of serum electrolyte concentrations
- ☐ D) Measurement of serum β -hCG concentration
- ☐ E) Thyroid function tests
- ☐ F) X-ray of the abdomen
- ☐ G) CT scan of the head



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22. A 23-year-old man is brought to the emergency department by police after he had to be removed in handcuffs from an airliner on which he had been a passenger. The airline attendants report that the patient shouted that he was a pilot and demanded to fly the plane, because he was too busy to wait for "those idiots that the airline calls pilots to take off." The patient reports, "I felt exquisite and perfectly glorious until those Keystone Cops started harassing me." He states that he has been brought to the hospital by "peons" in the past who failed to understand his essential superiority to "normal man." He says he has racing thoughts and boundless energy and has been sleeping only 2 hours nightly for the past week. He adds that he recently quit his job as a software engineer because he is now in complete control of the US government. He takes no medications. He has increased his alcohol consumption to six beers daily for the past 3 days. He does not use illicit drugs. He is wearing bright, mismatched clothing. Physical examination shows no abnormalities. On mental status examination, his mood is expansive and irritable, and his speech is loud and rapid. Which of the following is the most appropriate initial pharmacotherapy?

- ☐ A) Diazepam
- ☐ B) Fluoxetine
- ☐ C) Lamotrigine
- ☐ D) Quetiapine
- ☐ E) Sertraline



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23. A 20-year-old man is brought to the emergency department by his parents because he has not moved or spoken for 60 minutes despite encouragement. He has had similar episodes over the past 6 months. His parents say that he has always been shy and a loner. He has not looked for a job since graduating from high school 2 years ago. He appears unkempt and has long fingernails. He is mute and maintains a rigid posture during the examination. He resists passive movement of his extremities. Physical examination shows no other abnormalities. A CT scan of the head is normal. Which of the following is the most likely diagnosis?

- ☐ A) Absence seizures
- ☐ B) Brief psychotic disorder
- ☐ C) Narcolepsy
- ☐ D) Schizophrenia
- ☐ E) Selective mutism



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24. A previously well-functioning 14-year-old boy is apprehended for stealing a compact disc from a store. A court-ordered evaluation shows the stealing to be the latest in a series of self-defeating behaviors over the past 2 months that include incomplete homework assignments and truancy. He states that he has difficulty concentrating and awakens frequently at night. During an interview with his family, the teenager seems oblivious to the obvious marital discord between his parents. Which of the following is the most likely diagnosis?

- ☐ A) Adjustment disorder
- ☐ B) Antisocial personality disorder
- ☐ C) Conduct disorder
- ☐ D) Dysthymic disorder
- ☐ E) Post-traumatic stress disorder



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25. A 42-year-old man with obsessive-compulsive disorder comes to the physician because of only partial symptomatic relief with adequate trials of clomipramine and fluoxetine. He has fewer but persistent contamination obsessions and conducts fewer hand-washing rituals. He asks what else can be done to relieve his symptoms. Which of the following is the most appropriate next step in management?

- ☐ A) Seclusion and restraint
- ☐ B) SPECT scan of the brain
- ☐ C) Behavior therapy
- ☐ D) Hypnotic regression therapy
- ☐ E) Insight-oriented psychotherapy
- ☐ F) Buspirone therapy
- ☐ G) Clozapine therapy
- ☐ H) Electroconvulsive therapy



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26. A 32-year-old woman comes to the physician because of difficulty falling asleep for 3 years. She reports that it has been even more difficult to fall asleep during the past month. Five weeks ago, her cat of 4 years was hit by a car and died. She has a generally poor appetite but has not had any recent changes in weight or appetite. She has worked as a registered nurse on the overnight shift at a skilled nursing care facility for 5 years. She says that she has never missed a day of work in 3 years, although she rarely feels like getting out of bed. She states that she prefers night work because it involves less contact with families. She does not enjoy her job even though she is respected for her work and regularly receives good evaluations. She is neatly dressed. Her temperature is 36.8°C (98.2°F), pulse is 70/min, respirations are 10/min, and blood pressure is 120/65 mm Hg. Physical examination shows no other abnormalities. On mental status examination, she is pleasant and polite. She says she felt sad when her cat died but states that she cannot recall a time when her life did not seem sad to her. When asked about her social life, she says she has never been in a romantic relationship and states that "no one would really want me." Results of laboratory studies are within the reference range. Which of the following is the most appropriate next step in management?
- ☐ A) Reassurance that this is normal bereavement
 - ☐ B) Bupropion therapy
 - ☐ C) Buspirone therapy
 - ☐ D) Levothyroxine therapy
 - ☐ E) Modafinil therapy
 - ☐ F) Supplementation with a multivitamin and iron
 - ☐ G) Zaleplon therapy



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27. A 17-year-old girl is brought to the physician by her parents for a physical examination prior to participating in school sports. Her parents are concerned about her academic performance because she has received mostly B's and C's. Her parents and older brother attended Ivy League colleges. Every time they try to discuss applying to colleges, she becomes angry and tearful and goes to her room, slamming the door and refusing to talk about it. They try to talk to her through the closed door, but she will not cooperate. They have arranged alumni interviews for her at their own alma maters, and they are concerned that this negative attitude will impact her interviews. When they contacted her guidance counselor about their concerns and observations, he was surprised and reported that she has been diligently working on her applications with his help. She is at the 50th percentile for height and 30th percentile for weight. Physical examination shows no abnormalities. On mental status examination, she is cooperative but reticent. Her speech is articulate and coherent. Which of the following is the most likely explanation for these findings?

- ☐ A) Adjustment disorder with anxious mood
- ☐ B) Adjustment disorder with depressed mood
- ☐ C) Dysthymic disorder
- ☐ D) Mild mental retardation
- ☐ E) Oppositional defiant disorder
- ☐ F) Normal behavior



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28. A 72-year-old man who is a resident of a skilled nursing care facility is brought to the emergency department because of sweating and increasing confusion for 2 days. He underwent a below-the-knee amputation 4 weeks ago for osteomyelitis. He has a 15-year history of type 2 diabetes mellitus treated with glyburide. He also is receiving acetaminophen and codeine for pain, allopurinol for gout, and haloperidol for nighttime agitation. His temperature is 39.5°C (103.1°F), pulse is 120/min, and blood pressure is 130/90 mm Hg. Muscle tone is increased. The patient is too disoriented to respond appropriately to questions. Laboratory studies show:

Hematocrit	50%
Leukocyte count	8000/mm ³ with a normal differential
Serum	
Na ⁺	154 mEq/L
Glucose	175 mg/dL

Which of the following is the most likely cause of this patient's condition?

- ☐ A) Adverse effect of haloperidol
- ☐ B) Drug-drug interaction
- ☐ C) Exertional heat stroke
- ☐ D) Poor glucose control
- ☐ E) Septicemia

29. A 47-year-old woman comes to the physician to request a sedative to help calm her nerves. She has had extreme anxiety and difficulty eating and sleeping since she received the diagnosis of breast cancer 10 days ago. She is married and has two children, and she worries constantly about what will happen to them if she dies. She regularly volunteers at the school library, but she has been unable to go because of her anxiety since her diagnosis. She has not felt depressed, guilty, or suicidal. She has no other history of serious medical or psychiatric illness. There is no family history of psychiatric illness. Physical examination shows a healing incision over the breast biopsy site. On mental status examination, she is anxious but shows a full range of affect. Which of the following is the most likely diagnosis?

- ☐ A) Acute stress disorder
- ☐ B) Adjustment disorder
- ☐ C) Generalized anxiety disorder
- ☐ D) Major depressive disorder
- ☐ E) Psychological factors affecting medical condition
- ☐ F) Sedative-hypnotic abuse



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30. A 25-year-old woman is brought to the physician by her family 1 week after the sudden onset of bizarre behavior that began after she had a disagreement with her supervisor at work. The supervisor told her family that she became confused and began to accuse him of monitoring her phone calls and watching her every move because he wanted to fire her. Her work performance had been fine. She told her husband that she heard the voices of her coworkers talking about her when she was driving home from work 2 days ago. She used cocaine briefly 5 years ago. Physical examination shows no abnormalities. On mental status examination, she is oriented to person and place but not to date. She is unsure why she is being evaluated, and her answers to questions are incomprehensible at times. She admits to being worried and depressed about her coworkers being "after her." Laboratory studies are within normal limits, and toxicology screening shows traces of marijuana. Which of the following is the most likely diagnosis?

- ☐ A) Adjustment disorder with mixed disturbance of emotions and conduct
- ☐ B) Brief psychotic disorder
- ☐ C) Cocaine-induced psychotic disorder
- ☐ D) Post-traumatic stress disorder
- ☐ E) Schizophrenia



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31. A 3-year-old girl is brought to the physician by her parents because of behavior problems since starting preschool 3 months ago. Her parents report that she was adopted at 2 weeks of age and little is known about her biologic parents. Her teachers report that she is more active than her peers, is easily frustrated, and has frequent tantrums. She is not yet toilet trained. She is at the 20th percentile for height and weight. Physical examination shows a thin upper lip, a narrow philtrum, and bilateral palmar creases. Her gait is normal. On mental status examination, she speaks clearly, communicating in two- to three-word phrases. Which of the following is the most likely diagnosis?

- ☐ A) Attention-deficit/hyperactivity disorder
- ☐ B) Down syndrome
- ☐ C) Fetal alcohol syndrome
- ☐ D) Fragile X syndrome
- ☐ E) Rett disorder



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32. A 22-year-old college student comes to the physician because of depressed mood, restlessness, irritability, and an inability to concentrate since being raped 1 week ago. She has had difficulty sleeping because of recurrent nightmares about the rape and has not been able to study or attend classes. At times, she feels like the world around her is not real. Physical examination shows no abnormalities. On mental status examination, she is anxious and tense. She is unable to remember the details of the rape and becomes distressed and does not want to talk about it. Which of the following is the most likely diagnosis?
- ☐ A) Acute stress disorder
 - ☐ B) Brief psychotic disorder
 - ☐ C) Generalized anxiety disorder
 - ☐ D) Major depressive disorder
 - ☐ E) Post-traumatic stress disorder



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33. A 52-year-old man with cancer of the tongue comes to the physician 2 days after an episode of anxiety and panic while being fitted with a mask in preparation for radiation therapy. He reports similar symptoms during MRI of the brain; he says he was unable to tolerate being confined to such a small space and felt panic and the need to escape. On physical examination, the tongue is enlarged and tender. On mental status examination, he is anxious when describing being confined. The most appropriate pharmacotherapy for this patient targets which of the following neurotransmitters?

- ☐ A) Acetylcholine
- ☐ B) γ -Aminobutyric acid
- ☐ C) Dopamine
- ☐ D) Norepinephrine
- ☐ E) Serotonin



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34. A 47-year-old man comes to the physician because of decreased libido over the past 2 years. He describes his marriage as good. He is able to achieve erection only through masturbation. He does not smoke cigarettes or drink alcohol. He uses marijuana daily. His pulse is 72/min, respirations are 10/min, and blood pressure is 120/90 mm Hg. Physical examination shows no abnormalities. Serum studies show a thyroid-stimulating hormone concentration of 1 μ U/mL, thyroxine (T_4) concentration of 7 μ g/dL, and triiodothyronine (T_3) concentration of 125 ng/dL. Urine toxicology screening is positive for marijuana. Which of the following is the most appropriate next step in management?

- ☐ A) Recommendation to discontinue marijuana use
- ☐ B) Individual psychotherapy
- ☐ C) Marital therapy
- ☐ D) Levothyroxine therapy
- ☐ E) Sildenafil therapy



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35. An 18-year-old woman comes to the physician because she is anxious and concerned that she is contaminating her hands with HIV whenever she touches a door handle. She realizes that this is irrational but nevertheless has to wash her hands repeatedly with household bleach. She estimates that she washes her hands 10 times daily for 10 minutes at a time. Examination shows cracked and bleeding hands. Which of the following is the most appropriate pharmacotherapy?

- ☐ A) Bupropion
- ☐ B) Buspirone
- ☐ C) Fluoxetine
- ☐ D) Haloperidol
- ☐ E) Imipramine
- ☐ F) Lorazepam
- ☐ G) Propranolol



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36. Immediately after undergoing operative repair of a right knee dislocation, a 42-year-old woman has severe pain. She has a history of heroin addiction and has been receiving therapy through a community-based methadone maintenance program. Her methadone maintenance dose has remained constant for 2 years. She has no other history of medical or psychiatric illness. On examination, she rates her pain as a 10 on a 10-point scale, a level that is consistent with that reported by other patients who have undergone this operative procedure. Administration of which of the following is most appropriate at this time?
- ☐ A) A decreased methadone dose and additional analgesia on a set schedule
 - ☐ B) A decreased methadone dose and patient-controlled analgesia
 - ☐ C) An increased methadone dose and additional analgesia on a set schedule
 - ☐ D) The patient's usual methadone maintenance dose and patient-controlled analgesia
 - ☐ E) The patient's usual methadone maintenance dose only



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37. A 15-year-old boy is brought to the physician by his parents because of a 3-month history of social withdrawal and subtle but increasingly odd interpersonal interactions, such as blinking to avoid direct eye contact. He also has a 1-month history of intermittent daydreaming and episodes of talking to himself. His behavior was first noticed by his teacher and the school nurse, who notified his parents. The patient says that when in quiet locations, he hears vague, distorted sounds. He talks to himself to calm himself and to make the sounds disappear. He has no history of serious illness and takes no medications. His maternal uncle has bipolar disorder, and two paternal uncles have schizophrenia. The patient does not drink alcohol or use illicit drugs. Physical examination shows no abnormalities. On mental status examination, he is alert and fully oriented; he appears shy and has a flat affect. He says his mood is "okay." Further testing is most likely to show which of the following results in this patient?

- ☐ A) Abnormal amyloid precursor protein
- ☐ B) Decline in working memory
- ☐ C) Decreased auditory acuity
- ☐ D) Decreased visual fields
- ☐ E) Hyperphosphorylation of the Tau protein



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38. Three weeks after voluntarily entering a residential treatment facility for alcoholism, a 27-year-old woman is being prepared for discharge. She requests medication to control her urge to drink alcohol and decrease her risk for relapse. Prior to admission, she drank 6 to 12 glasses of wine nightly. She tried to quit drinking several times but says her urges to drink returned after 2 to 3 days of abstinence. She has no other history of serious illness and takes no medications. Physical and mental status examinations show no abnormalities. Which of the following is the most appropriate pharmacotherapy?
- ☐ A) Buprenorphine
 - ☐ B) Bupropion
 - ☐ C) Levomethadyl acetate
 - ☐ D) Naloxone
 - ☐ E) Naltrexone



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39. As a condition of employment, a newly hired police officer has agreed to avoid the use of all illicit drugs both on and off duty and to provide random urine samples for drug screening. Although he has not used any drugs in the past month, his urine sample shows a definite positive reaction. Which of the following drugs would most likely account for this finding?

- ☐ A) Cocaine
- ☐ B) Heroin
- ☐ C) LSD
- ☐ D) Marijuana
- ☐ E) Methamphetamine



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40. A 19-year-old man is brought to the physician by his parents because of changes in his behavior during the past 6 months. During this time, he has been obsessed with what he calls "the ultimate truth of the universe" and has spent most of his time in his room alone. His parents say that he has always been serious, curious, and quiet. The few friends he had stopped interacting with him because of this behavior. The patient explains that he needs to solve "the ultimate and consummate algorithmic result." He has no history of serious illness. He does not drink alcohol or use illicit drugs. He is 173 cm (5 ft 8 in) tall and weighs 98 kg (217 lb); BMI is 33 kg/m². His pulse is 82/min, and blood pressure is 116/77 mm Hg. Physical examination shows abdominal striae. On mental status examination, he describes his mood as "transfixed." He has a blunt affect. His thought process is disorganized, and he continues to talk about the equation. There is no evidence of suicidal ideation. Results of laboratory studies are within the reference ranges. Urine toxicology screening is negative. A CT scan of the head shows no abnormalities. Which of the following is the most appropriate pharmacotherapy?

- ☐ A) Buspirone
- ☐ B) Clonazepam
- ☐ C) Clozapine
- ☐ D) Fluoxetine
- ☐ E) Lithium carbonate
- ☐ F) Ziprasidone



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41. A 42-year-old man comes to the physician for a routine health maintenance examination for insurance purposes. When asked for an emergency contact, he gives the name of his mother but says that she lives in another state and that he has not seen her for 5 years. He lives by himself and says that he has no friends. He spends most of his free time alone in his apartment cutting out articles about animals from magazines and newspapers that he plans to use in the future. He has worked the graveyard shift of his company for 10 years with few absences. On two occasions, he was offered a day shift but refused. He has no history of serious illness and takes no medications. He does not drink alcohol or use illicit drugs. He is wearing mismatched socks, and his glasses are tied to a piece of twine he wears around his neck. Physical examination shows no abnormalities. On mental status examination, he is alert and oriented to person, place, and time. His thoughts are goal directed but difficult to understand because of his frequent use of idiosyncratic metaphors. He reports no auditory or visual hallucinations. He can recall three of three objects after 5 minutes and describes events of the past several years without difficulty. Which of the following is the most likely diagnosis?

- ☐ A) Agoraphobia
- ☐ B) Antisocial personality disorder
- ☐ C) Avoidant personality disorder
- ☐ D) Obsessive-compulsive disorder
- ☐ E) Schizophrenia
- ☐ F) Schizotypal personality disorder
- ☐ G) Social phobia



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42. A 47-year-old man comes to the physician because of a 2-week history of anxiety and difficulty falling asleep. He attributes his symptoms to the stress related to his 21-year-old daughter's wedding plans. He divorced his wife 11 years ago because he felt more sexually attracted to men than women. He has had a monogamous homosexual relationship for 7 years. He relates that he is happier now than when he was married, but he has been having some doubts recently. He would like to bring his partner to the wedding and accompany his daughter down the aisle, but the groom's parents are protesting his attendance. Which of the following is the most likely diagnosis?

- ☐ A) Adjustment disorder with anxiety
- ☐ B) Exhibitionism
- ☐ C) Gender identity disorder
- ☐ D) Generalized anxiety disorder
- ☐ E) Transsexualism



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43. A 67-year-old woman with major depressive disorder is brought to the physician by her daughter for a follow-up examination. During the past 6 months, she has received trials of citalopram, fluvoxamine, and escitalopram with no improvement in her symptoms. Four weeks ago, she was switched from fluoxetine to sertraline therapy because the fluoxetine caused excessive tiredness. Her daughter reports that the patient is "slightly better, but not dramatically so." She has had a decreased appetite resulting in a 9-kg (20-lb) weight loss during the past 3 months. Today, her eyes are closed throughout most of the visit. She appears cachectic and disheveled. She is 163 cm (5 ft 4 in) tall and weighs 43 kg (95 lb); BMI is 16 kg/m². Physical examination shows poor skin turgor and severe psychomotor retardation. On mental status examination, she responds to few of the physician's questions and uses only brief responses. There is no evidence of suicidal ideation or psychotic symptoms. Which of the following is the most appropriate next step in management?

- ☐ A) Add bupropion to her current medication regimen
- ☐ B) Add olanzapine to her current medication regimen
- ☐ C) Switch from sertraline to nortriptyline
- ☐ D) Switch from sertraline to paroxetine
- ☐ E) Admit to the psychiatric unit



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44. A 25-year-old man is admitted to the hospital because of extreme hyperactivity for 2 days. He is unable to conduct an intelligent conversation because of flight of ideas. He is started on haloperidol therapy (10 mg daily). Two days later, he has difficulty breathing, uncontrollable tightening of his face and neck muscles with spasm, and a rolling up and locking of his eyes. Which of the following is the most appropriate pharmacotherapy?

- ☐ A) Bethanechol
- ☐ B) Chlordiazepoxide
- ☐ C) Clonazepam
- ☐ D) Diphenhydramine
- ☐ E) Propranolol



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45. A 27-year-old man is brought to the emergency department by police after he threatened to commit suicide in jail. He was arrested 3 days ago for the murder of his girlfriend. Two years ago, he completed a 5-year prison sentence for assault. He was treated in a psychiatric hospital for conduct disorder at the age of 15 years. He has no history of serious illness and takes no medications. He has a history of drinking excessive amounts of alcohol and using illicit drugs. He is cooperative with the hospital staff but becomes irritable when the police officer who is guarding him attempts to interact with him. His temperature is 36.6°C (97.8°F), pulse is 78/min, and blood pressure is 132/82 mm Hg. Physical examination shows no abnormalities except for multiple scars over the face. On questioning, he says that he got the scars in fights. On mental status examination, there is no evidence of depression or psychosis. His speech is clear and slow, and he says, "If you don't release me and I am sent back to jail, I will find a way to kill myself!" Metabolites of which of the following neurotransmitters are most likely to be decreased in this patient's cerebrospinal fluid?

- ☐ A) Acetylcholine
- ☐ B) γ -Aminobutyric acid
- ☐ C) Dopamine
- ☐ D) Norepinephrine
- ☐ E) Serotonin



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46. A 19-year-old woman is brought to the emergency department after she became agitated and disruptive on an airplane. She was screaming and using profanity, and the plane was forced to land. On arrival, she is agitated and hypervigilant, and her speech is rapid and pressured. Her temperature is 37.2°C (99°F), pulse is 120/min, respirations are 16/min, and blood pressure is 140/90 mm Hg. Examination shows warm dry skin. The pupils are 8 mm bilaterally. Which of the following is the most likely diagnosis?

- ☐ A) Acetaminophen overdose
- ☐ B) Anticholinergic delirium
- ☐ C) Benzodiazepine overdose
- ☐ D) Hypoglycemia
- ☐ E) Narcotic overdose
- ☐ F) Pulmonary embolism



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47. A 17-year-old girl is brought to the emergency department after attempting suicide by stabbing herself. Her father died suddenly 1 week ago, and she states that she heard his voice telling her to join him. Her mother reports that the patient became wildly agitated yesterday while visiting her father's grave. After arriving home, she sat alone in a corner, rocked back and forth, and talked about "the Devil" taking her soul. Her mother says that she does well in school and has many friends. She has no history of medical or psychiatric illness. Physical examination shows a self-inflicted superficial stab wound over the abdomen. On mental status examination, she talks about seeing angels and devils. Which of the following is the most likely explanation for these findings?

- ☐ A) Adjustment disorder with mixed disturbance of emotions and conduct
- ☐ B) Bereavement
- ☐ C) Brief psychotic disorder
- ☐ D) Major depressive disorder with psychotic features
- ☐ E) Schizoaffective disorder
- ☐ F) Separation anxiety disorder



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48. An 82-year-old man is brought to the physician by his daughter because of progressive problems with his memory during the past 3 years. She became concerned when his electricity was turned off recently after he failed to pay the bill. The patient says that he is not concerned about his memory and that his daughter worries too much. He has no history of serious medical or psychiatric illness. Vital signs are within normal limits. Physical examination, including neurologic examination, shows no abnormalities. On mental status examination, he has a euthymic mood and reactive affect. He makes numerous paraphasic errors, and his thought process is circumstantial. He recalls one of three words after 5 minutes. He is able to remember his date of birth, the name of his high school, and the year he was married. The patient's drawing of the intersecting triangles in Figure 1 is shown in Figure 2. His Mini-Mental State Examination score has decreased from 26/30 1 year ago to 23/30 today. This patient's cognitive disorder is most likely associated with mutations in genes encoding which of the following?

- ☐ A) Huntingtin
- ☐ B) Prion
- ☐ C) Superoxide dismutase
- ☐ D) α -Synuclein
- ☐ E) *TAU*

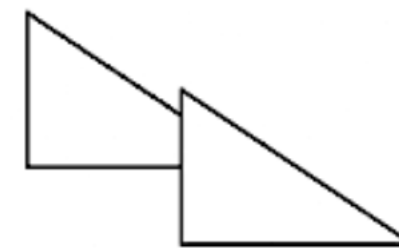


Figure 1



Figure 2

49. A 52-year-old man with angina pectoris comes for a follow-up examination. His wife reports that he is concerned that his neighbors have been watching and videotaping him over the past 3 months. He has no history of these concerns prior to this time. He has suggested to his wife that they build a 14-foot fence to stop this surveillance, but she is not concerned that they are being videotaped. He believes that the neighbors might have followed him to his appointment today, but he does not believe that they would come into the office and speak to the office personnel. His medications include metoprolol and aspirin. Physical examination shows no abnormalities. On mental status examination, he is guarded and hypervigilant. He is oriented to person, place, and time. Which of the following is the most likely diagnosis?

- ☐ A) Brief psychotic disorder
- ☐ B) Delusional disorder
- ☐ C) Mood disorder due to a general medical condition
- ☐ D) Schizotypal personality disorder
- ☐ E) Substance-induced psychotic disorder



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50. A 19-year-old college sophomore comes to the physician because of episodes of shortness of breath, palpitations, and sweating that have awakened him from sleep several times over the past 2 months. He says that the episodes are so alarming that he thinks he is going to die and is afraid that he has a serious illness. He reports episodes of sadness since he broke up with his girlfriend 3 weeks ago. He does not smoke, drink alcohol, or use illicit drugs. He requests a bronchodilator inhaler. His pulse is 80/min, respirations are 20/min, and blood pressure is 100/70 mm Hg. The lungs are clear to auscultation. Physical examination shows no abnormalities. Which of the following is the most likely diagnosis?

- ☐ A) Adjustment disorder with anxiety
- ☐ B) Factitious disorder
- ☐ C) Generalized anxiety disorder
- ☐ D) Panic disorder
- ☐ E) Post-traumatic stress disorder
- ☐ F) Separation anxiety disorder
- ☐ G) Somatization disorder



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